



Discharge follow up for mental health inpatients

Reference #: FSH-MENH-POL-0019

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital Cockburn Health	Mental Health Services	Medical, Nursing, Allied Health, Clerk

1. Introduction

Discharge from hospital is a critical transition point in the delivery of mental health care. People leaving hospital after an admission for an episode of mental illness have increased vulnerability. Diligent follow up may help avoid relapse and readmission. This document outlines the Mental Health Services process for appropriately following up patients with a follow up contact within 7 days of discharge, including for those patients who do not keep their planned follow up arrangements.

2. Terminology

Acute mental health inpatient service	A service that provides voluntary and involuntary short-term inpatient management and treatment during an acute phase of mental illness, until the patient has recovered enough to be treated effectively and safely in the community ¹ . Includes Hospital in the Home teams.
Personal Support Persons (PSP)	Can be any of the following people who are supporting someone who is experiencing mental illness – close family member, significant other, carer, nominated person, the parent or guardian of a child and any guardian or enduring guardian of an adult ² .
Psychiatric Services On-line Information System (PSOLIS)	Mental health clinical information system used by all government mental health agencies throughout Western Australia.
PSOLIS Service event	A service to, or on behalf of, a patient including one or more of the following: • actual service contact delivered

- must involve at least two persons
- one involved must be a professional

Contact can occur face-to-face, via telephone or video.

3. Policy

- All patients who have had an overnight admission to a mental health unit must receive a contact where the patient and/or PSP participate in the seven days immediately following their discharge¹.

The following patients are not required to have community follow up within seven days:

- Same day admissions
- Discharge/transfers to another acute or mental health unit
- Discharge/transfers to a residential mental health service e.g., Hampton House
- Separations where the length of stay is equivalent to one day for Electroconvulsive Therapy or Transcranial Magnetic Stimulation
- Statistical and change of care type discharges
- Separations that end in death

- Prior to discharge patients, and where relevant their PSP, will be informed that they will receive a follow up phone call after discharge
- The follow up phone call will be made by an appropriately experienced clinician and include review of the patient's mental state and current risks.
- Up to three attempts will be made to contact the patient by telephone over the seven-day period.
- All contacts and/or attempts to contact the patient must be documented in PSOLIS as a service event and in the patient's digital medical record (DMR).

Where a patient is accepted by a Community Treatment Team (CTT) from inpatient care, governance of clinical care will be transferred to the CTT on discharge from the ward.

4. FSH and Cockburn Health Procedure

4.1. Follow up phone call

- The Ward Clerk will print a daily list of inpatients discharged the day prior from webPAS.

- The Ward Clerk will add the patient to the 7-day Discharge Follow up Log Sheet/excel spreadsheet for each ward including the 7 day follow up due date
- The Inpatient Team will allocate a member of the treating team to complete 7-day follow up
- The allocated clinician will:
 - phone the patient
 - document on the Follow Up Log Sheet/excel spreadsheet when follow up is complete
 - document “conversation or attempt to call” in the DMR in the relevant inpatient episode with heading: “7 day discharge F/U”
 - enter all contacts and/or attempts to contact the patient as a service event directly into PSOLIS
 - complete the required fields on 7 Day Discharge Follow up Log Sheet

4.2. Escalation of care

- If unable to contact the patient and/or PSP after three attempts the allocated clinician will:
 - inform the Treating Team consultant
 - contacting or referring to the relevant community contact (CTT, ATT or GP)

5. FH Procedure

5.1. Follow up phone call

- Attempts will be made to make direct contact (face- to-face or by phone) with the patient the day following discharge from an inpatient unit.
- The Inpatient Registrar is responsible for ensuring that a follow up phone call occurs within 24 hours of discharge.
- The Inpatient Registrar will make the follow up phone call or:
 - refer the patient to the Assessment and Treatment Team (ATT) for follow up on a weekend. The referral must include a completed Risk Assessment and Management Plan (RAMP) and indicate the steps to be taken by ATT in the event they are unable to contact the patient by phone.
 - delegate responsibility to another inpatient junior medical officer
- In the event the Inpatient Registrar is not on duty within the 24-hour timeframe the Inpatient Team Consultant will be informed, and the responsibility delegated to another inpatient clinician.
- The Inpatient Registrar will:

- communicate the outcome of the phone call to the patient's Care Coordinator (where relevant)
- document the post discharge service event in PSOLIS.

5.2. Outpatients follow up appointment

- All patients discharged from the inpatient ward will be booked to have a face-to-face outpatient appointment within seven days of discharge
- The Inpatient Registrar/Intern will:
 - Contact the Outpatient Clerk to book an outpatient appointment with the discharging Registrar within seven days of discharge
 - Inform the patient and/or PSP of the appointment date/time and provide an appointment card.
- On discharge from the ward the Ward Clerk will ensure the patient has an active referral on PSOLIS.
- All patients with an allocated non-medical Care Coordinator will be reviewed face-to-face by their Care Coordinator within 72 hours of discharge where possible
- All contacts and/or attempts to contact the patient will be entered as a service event directly into PSOLIS.
- The Outpatient Clerk provides a webPAS report to the inpatient treating teams listing all patients who have a seven day follow up appointment scheduled the following week.

5.3. Older adults

- The older adult inpatient unit is responsible for the follow up of patients discharged:
 - outright with no referral to another mental health service
 - out of area and accepted by the patient's local mental health service
- Fremantle Older Adult Mental Health Service (FOAMHS) is responsible for the follow up of:
 - all active community patients discharged from the inpatient ward
 - all patients referred to the FOAMHS on discharge from the older adult ward.

5.4. Escalation of care

5.4.1. Where the clinician is unable to contact the patient and/or PSP at the 24-hour phone call care will be appropriately escalated:

- The Clinician responsible for making the follow up call will:

- inform the Inpatient Treating Team Consultant or their delegate at the daily ward handover or Multidisciplinary Team meeting
 - inform the patient's Care Coordinator and/or Community Treatment Team (CTT) of unsuccessful phone contact with the patient
 - The on-call Consultant Psychiatrist will be the delegate for the Inpatient Treating Team Consultant after hours/ weekends
 - The urgency of response will be determined based on the patient's risk assessment, clinical judgement of the reviewing team and communication between the inpatient team and the community team.
 - The Clinician will refer the patient to the CTT, ATT or General Practitioner for follow up as indicated.
 - All risk assessments, escalation of care and actions taken to follow up a patient will be documented in the patient's medical record.
- 5.4.2. If the patient does not attend their follow up appointment staff will follow the steps outlined in the "FSFHG Did not attend: Mental Health outpatient appointments" procedure.

6. Compliance/Performance Monitoring

- The Mental Health Service will monitor compliance with target >75% monthly at the Service 5 Performance Management Committee and the SMHS Board.

7. Related Policy Documents

- WA Health:
 - [Missing Person policy MP-0012/16](#)
 - Safety Planning for Mental Health Consumers
 - [Clinical Care of People who May be Suicidal MP-0074/17](#)
- SMHS:
 - [Care coordination in mental health](#)
 - [Missing or suspected missing mental health patients](#)
- FSFHG:
 - Clinical Documentation (FSFH-HW-POL-0039)
 - Clinical Handover (FSFH-HW-POL-0035)
 - Nursing staff escalation process – Mental Health (FSH-MENH-GUI-0005)
 - Escalation process for Nursing staff (FSFH-HW-POL-0014)

- Did not attend mental health outpatient appointments (FSFH-MENH-PRO-0001)
- Assessment and Treatment Team Referral for out of hours follow-up (FSFH-MENH-PRO-0011)
- Community Mental Health Status Assessment (FSFH-MENH-PRO-0006)

8. Other Related Document

- [The Mental Health Act 2014](#)

9. Standards

- NSQHS
 - Clinical Governance
 - Comprehensive Care
- Chief Psychiatrists Standards for Clinical Care
 - Transfer of Care

10. References

1. Government of Western Australia, Department of Health. Health Service Performance Report Performance Indicator Definition Manual. 2022-23 Version 1.1.
2. Chief Psychiatrists Clinicians' Practice Guide to the Mental Health Act 2014 Edition 3. November 2015.

11. Authorisation

EXECUTIVE SPONSOR: Co Director Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	02/2016	Mental Health Service Nurse Director	MHS Executive Committee	Policy Committee	02/2019
2	06/2017	Mental Health Service Nurse Director	Mental Health Clinical Practice Committee	FSFHG Policy Committee	06/2020
3	08/2019	Programme Manager, Adult, MHS	MHS SQRC	FSFHG Policy Committee	06/2020
4	12/2022	CNS Clinical Improvement	MHS SQRC	FSFHG Policy Committee	12/2022
5	02/2023	CNS Clinical Improvement		Service 5 Safety Quality and Risk Committee	02/2025
5.1	06/2023	CNS Clinical Improvement	Minor update formatting error	No change to review date	06/2025
6	12/2023	CNS Clinical Improvement	MHS SQRC	Service 5 Safety Quality and Risk Committee	01/2027
6.1	07/2024	S5 SQR Lead	Minor update, No change to review date	Service Director – S5	01/2027